

COMMUNITY NEEDS ASSESSMENT

H.Y.P.E. to Empower

HIV Health & Linkage Services
Gwinnett, Newton and Rockdale Counties
June 2026

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1.1 Purpose and Background

This community needs assessment was conducted to characterize the HIV-related health needs, existing community assets, and service gaps among Black/African American individuals, Hispanic/Latino populations, women of color, LGBTQ+ individuals, young adults, and persons at risk for HIV in Gwinnett, Newton & Rockdale Counties. The findings will inform program planning and evaluation activities at H.Y.P.E. to Empower and support the organization's capacity for data-driven decision-making.

1.2 Target Population and Geographic Focus

Table 1. Target Population and Geographic Focus

Category	Description
Target Population	Black/African American individuals, Hispanic/Latino populations, women of color, LGBTQ+ individuals, young adults, and persons at risk for HIV
Geographic Focus	Gwinnett, Newton & Rockdale Counties, Georgia
Key Demographics	Diverse population with substantial Black and Hispanic communities and documented HIV disparities
Health Issue	HIV prevention, testing, linkage-to-care, and health equity

1.3 Data Sources and Methods

The Community Needs Assessment utilizes publicly available surveillance, epidemiological, demographic, and mortality data to characterize HIV burden, disparities, and prevention needs within Gwinnett, Newton & Rockdale Counties.

HIV Surveillance and Epidemiological Data

- **CDC AtlasPlus** – HIV surveillance data including HIV prevalence, new HIV diagnoses, demographic characteristics, and prevention indicators. Available at: <https://gis.cdc.gov/grasp/nchhstpatlas/tables.html>. Retrieved June 2026.
- **AIDSVu** – County-level HIV prevalence, new HIV diagnoses, linkage-to-care, and PrEP coverage data. Available at: <https://aidsvu.org>. Retrieved June 2026.
- **Georgia Department of Public Health (GDPH) OASIS** – HIV/AIDS mortality data and population health indicators for Gwinnett County. Available at: <https://oasis.state.ga.us>. Retrieved June 2026.
- **CDC HIV Surveillance Reports** – National HIV epidemiology, surveillance trends, and Ending the HIV Epidemic (EHE) benchmarks. Available at: <https://www.cdc.gov/hiv-data>. Retrieved June 2026.

Demographic and Socioeconomic Data

- **U.S. Census Bureau QuickFacts** – Population, race/ethnicity, socioeconomic, health insurance, and demographic characteristics for Gwinnett County, Georgia. Available at: <https://www.census.gov/quickfacts/gwinnettcountrygeorgia>. Retrieved June 2026.

Supporting Literature and Reports

- Supporting literature was reviewed to provide context regarding HIV disparities, linkage-to-care outcomes, PrEP utilization, stigma, social determinants of health, and community-based HIV prevention strategies among priority populations. Research consistently demonstrates that HIV disproportionately affects Black/African American individuals, Hispanic/Latino populations, gay, bisexual, and other men who have sex with men (GBMSM), and transgender communities, particularly within the Southern United States. Studies have identified structural barriers such as poverty, healthcare access challenges, stigma, discrimination, and social inequities as important contributors to HIV disparities and gaps in prevention and care (Arrington-Sanders et al., 2020; Furukawa et al., 2020; Hussen et al., 2024; Mann et al., 2022).
- Literature also highlights the critical role of community-based organizations in increasing access to HIV testing, prevention education, PrEP services, linkage-to-care, and patient navigation support. Evidence suggests that culturally responsive and community-centered approaches can improve engagement in care and reduce barriers among populations disproportionately affected by HIV (Piske et al., 2024).
- Federal guidance and surveillance reports were reviewed to inform the assessment, including the National HIV/AIDS Strategy 2022–2025, CDC HIV Surveillance Reports, and Ending the HIV Epidemic (EHE) initiative resources (U.S. Department of Health and Human Services, 2022; CDC, 2023). Literature and supporting reports were used to contextualize local findings, identify evidence-based HIV prevention strategies, and inform recommendations for future program planning, service delivery, and evaluation activities. A complete literature review is provided in Appendix A.

Table 2. Data Sources Used in the Community Needs Assessment

Data Source	Purpose
CDC AtlasPlus	HIV prevalence, diagnoses, demographic trends
AIDSVu	County-level HIV prevalence, linkage-to-care, PrEP coverage
Georgia Department of Public Health	HIV/AIDS mortality and surveillance data
U.S. Census Bureau ACS	Demographic and socioeconomic characteristics
Peer-Reviewed Literature	Evidence-based HIV prevention and disparity findings
CDC HIV Surveillance Reports	National benchmarks and epidemiological context

Methods

This Community Needs Assessment utilized a secondary data analysis approach to examine HIV burden, disparities, prevention indicators, and community health needs within Gwinnett, Newton & Rockdale Counties.

Data on HIV prevalence, new HIV diagnoses, linkage-to-care outcomes, PrEP coverage, and HIV-related mortality were extracted from CDC AtlasPlus, AIDSVu, and Georgia Department of Public Health surveillance systems. Demographic and socioeconomic information was obtained from the U.S. Census Bureau American Community Survey.

Data were analyzed using descriptive statistics and trend analysis to:

- Characterize the local HIV epidemic.
- Assess HIV-related disparities among racial and ethnic populations.
- Examine trends in HIV prevalence, diagnoses, linkage-to-care, and PrEP utilization.
- Identify populations disproportionately affected by HIV.
- Assess structural and social factors influencing HIV prevention and care access.

Findings from the secondary data review were synthesized to identify priority populations, service gaps, and opportunities for HIV prevention, testing, linkage-to-care, and community outreach efforts. These findings informed the development of the H.Y.P.E. to Empower Health & Linkage Services Program Evaluation Plan.

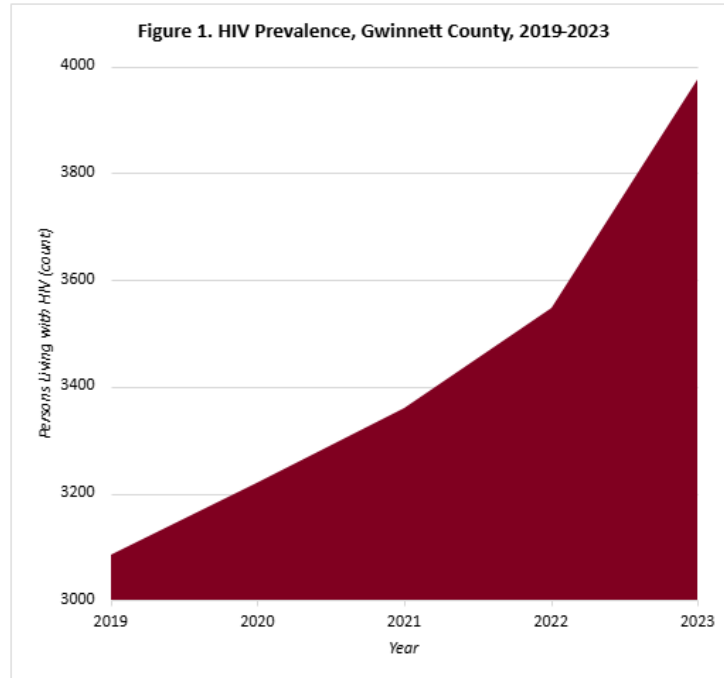
1.4 Key Findings

A. HIV Burden in Gwinnett County

Secondary surveillance data indicate that HIV remains a significant public health concern within Gwinnett County. Although annual HIV diagnoses have fluctuated over time, the number of individuals living with HIV continues to increase, reflecting both ongoing transmission and improved survival among people living with HIV.

Table 3. HIV Burden in Gwinnett County

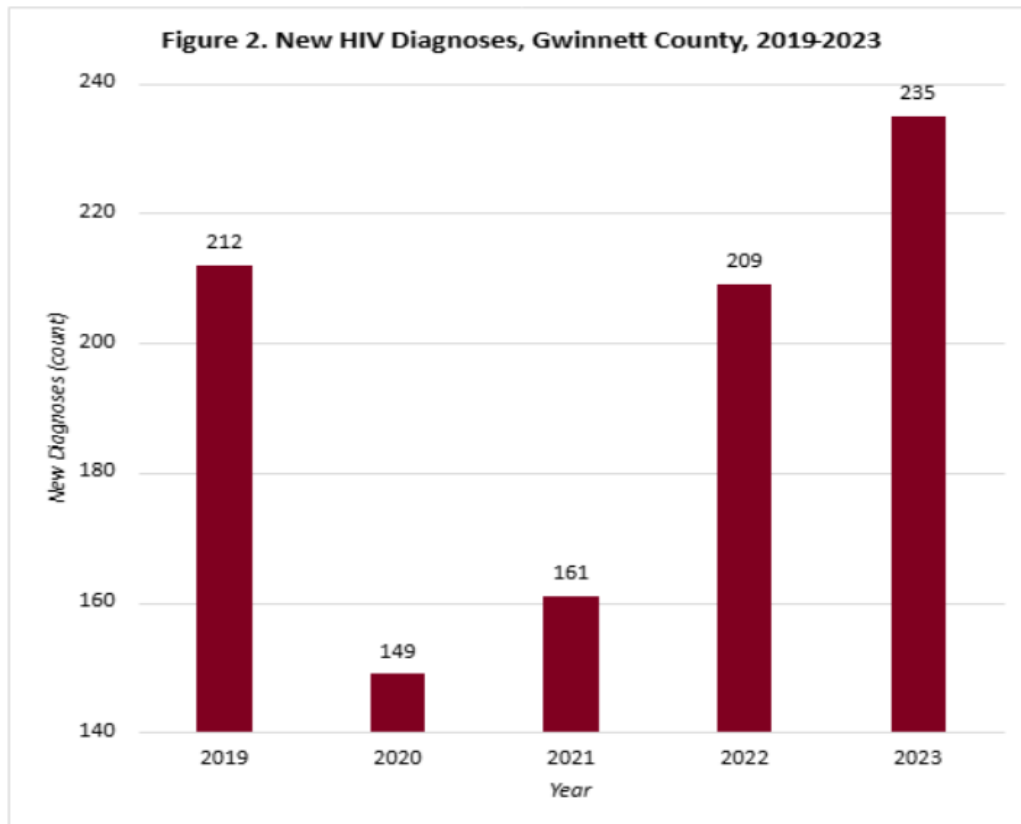
Indicator	Most Recent Year	Cases	Rate per 100,000
HIV Prevalence	2023	3,976	489.8
New HIV Diagnoses	2023	235	29.0



Source: CDC AtlasPlus (2025); AIDSVu (2025).

Table 4. New HIV Diagnoses Trend, Gwinnett County (2019–2023)

Year	New HIV Diagnoses	Rate per 100,000
2019	212	27.8
2020	149	19.0
2021	161	20.3
2022	209	26.0
2023	235	29.0



Source: CDC AtlasPlus (2025); AIDSVu (2025).

Key Findings

The number of people living with HIV increased from 3,085 individuals in 2019 to 3,976 individuals in 2023, representing a 28.9% increase over the five-year period. HIV prevalence rates also increased from 404.1 per 100,000 population in 2019 to 489.8 per 100,000 population in 2023.

New HIV diagnoses declined during 2020, likely reflecting disruptions in testing and healthcare access during the COVID-19 pandemic, but subsequently increased to 235 diagnoses in 2023. The diagnosis rate reached 29.0 per 100,000 population in 2023.

Implications

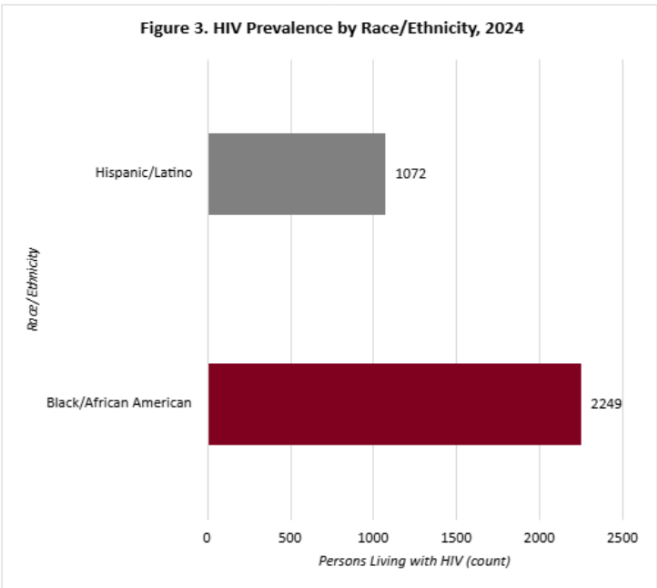
These findings indicate an ongoing HIV burden within Gwinnett County and demonstrate continued need for HIV prevention, testing, linkage-to-care, and community outreach services. The increasing prevalence of HIV highlights the importance of sustained prevention efforts and long-term support for people living with HIV.

B. Health Disparities

Analysis of HIV prevalence and diagnosis data revealed substantial racial and ethnic disparities within Gwinnett County.

Table 5. HIV Prevalence and Diagnosis by Race/Ethnicity

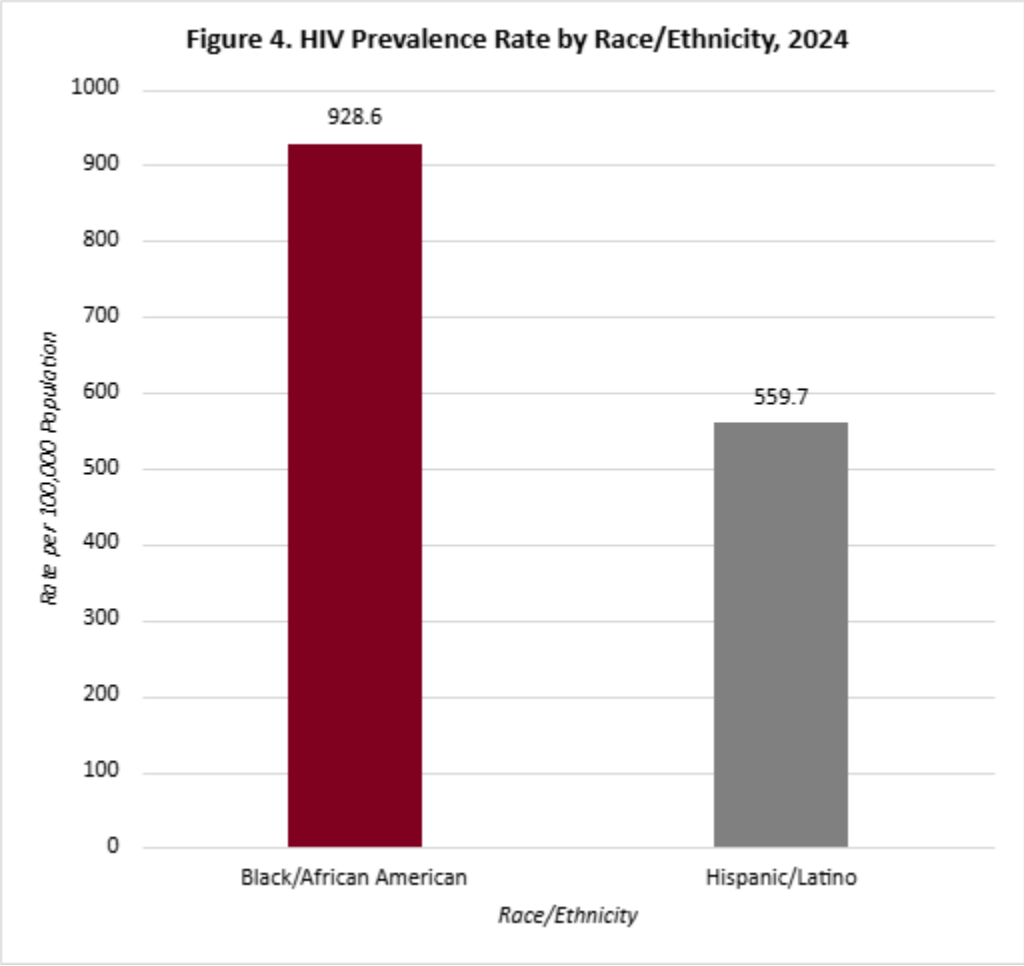
Race/Ethnicity	HIV Prevalence Cases	Prevalence Rate	HIV Diagnoses Cases	Diagnosis Rate
Black/African American	2,249	928.6	95	39.2
Hispanic/Latino	1,072	559.7	92	48.0



Source: CDC AtlasPlus (2025)

Table 6. HIV Prevalence Rates by Race/Ethnicity

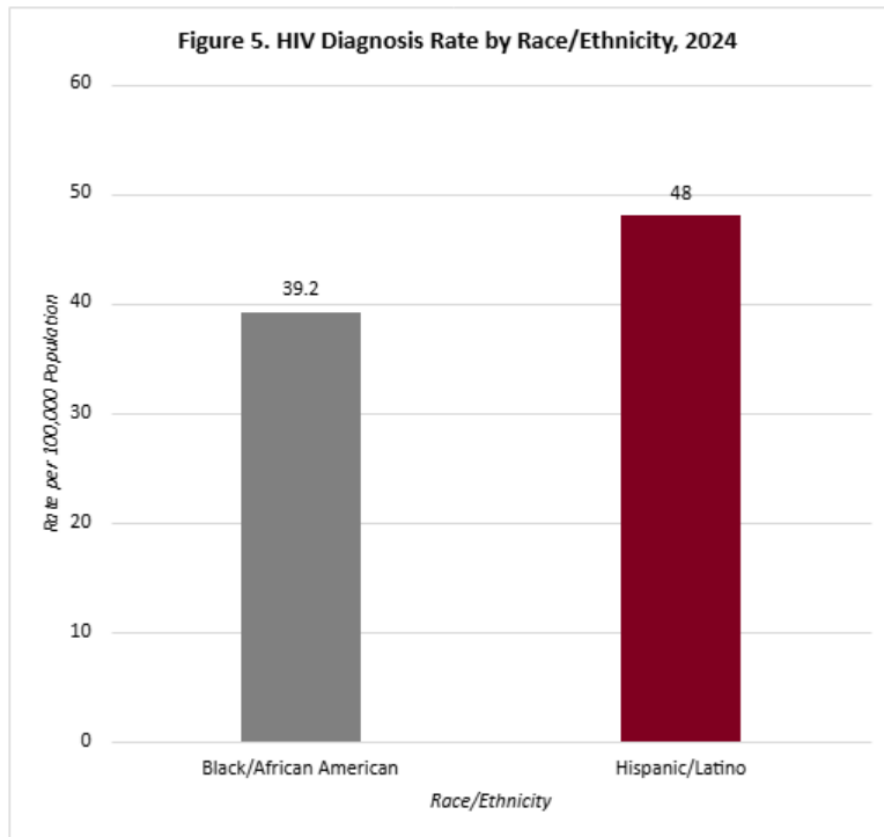
Race/Ethnicity	Rate per 100,000
Black/African American	928.6
Hispanic/Latino	559.7



Source: CDC AtlasPlus (2025)

Table 7. HIV Diagnosis Rates by Race/Ethnicity

Race/Ethnicity	Cases	Rate per 100,000
Black/African American	95	39.2
Hispanic/Latino	92	48.0



Source: CDC AtlasPlus (2025)

Key Findings

Black/African American residents experienced the greatest HIV burden within Gwinnett County, accounting for 2,249 people living with HIV and a prevalence rate of 928.6 per 100,000 population. Hispanic/Latino residents accounted for 1,072 people living with HIV and a prevalence rate of 559.7 per 100,000 population.

New HIV diagnoses also remained elevated among both populations. In 2024, diagnosis rates reached 39.2 per 100,000 among Black/African American residents and 48.0 per 100,000 among Hispanic/Latino residents.

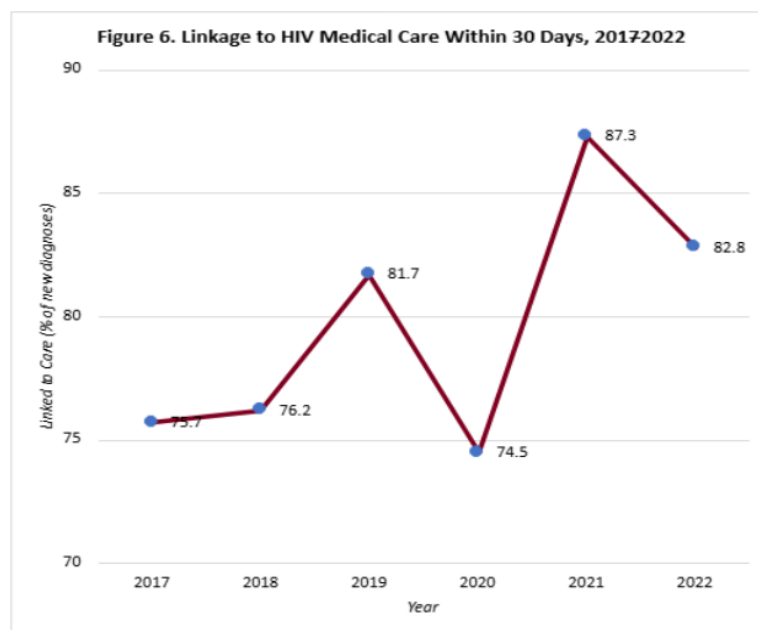
Implications

These findings demonstrate persistent racial and ethnic disparities in HIV burden and indicate that Black/African American and Hispanic/Latino communities continue to experience disproportionate impacts from HIV. These populations should remain priority populations for HIV prevention, testing, education, navigation, and linkage-to-care efforts.

C. Linkage to HIV Medical Care

Table 8. Linkage to HIV Medical Care Within 30 Days (2017–2022)

Year	Linkage to Care (%)
2017	75.7
2018	76.2
2019	81.7
2020	74.5
2021	87.3
2022	82.8



Source: AIDSVu (2025).

Key Findings

Linkage to HIV medical care improved overall between 2017 and 2022, increasing from 75.7% to 82.8%. The highest linkage rate occurred in 2021 at 87.3%, followed by a slight decline in 2022.

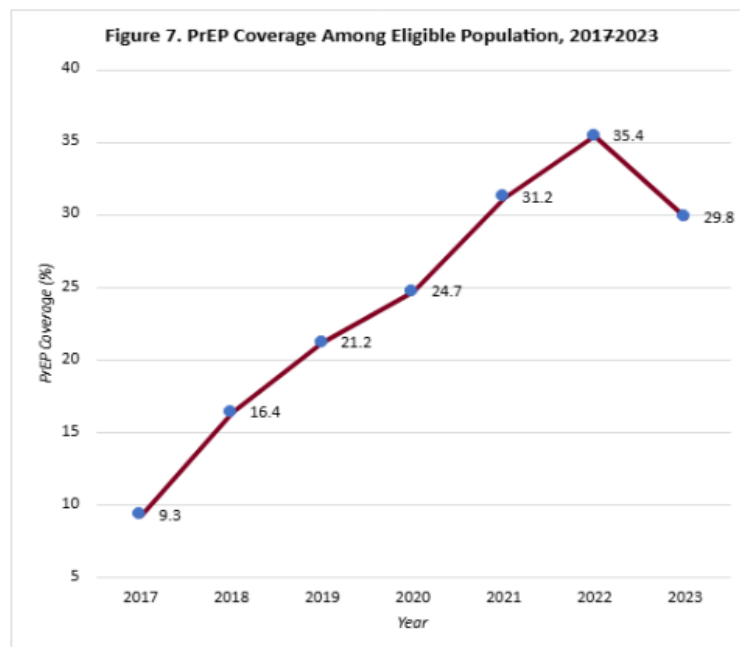
Implications

Although linkage-to-care outcomes improved over time, the most recent rate remains below the Ending the HIV Epidemic target of 95%. These findings suggest opportunities to strengthen referral coordination, patient navigation, and support services that facilitate timely engagement in HIV medical care following diagnosis.

D. PrEP Coverage

Table 9. PrEP Coverage Among Eligible Population (2017–2023)

Year	PrEP Coverage (%)
2017	9.3
2018	16.4
2019	21.2
2020	24.7
2021	31.2
2022	35.4
2023	29.8



Source: AIDSVu (2025).

Key Findings

PrEP coverage increased substantially between 2017 and 2022, rising from 9.3% to 35.4%. However, coverage declined to 29.8% in 2023.

Implications

Although PrEP utilization has improved over time, coverage remains insufficient relative to the ongoing HIV burden within the county. Continued expansion of PrEP education, awareness campaigns, navigation services, and healthcare access initiatives may help increase utilization among populations at elevated risk for HIV.

E. Demographic Context

Gwinnett County is one of the largest and most diverse counties in Georgia, with an estimated population of 1,018,099 residents in 2025. The county has experienced substantial population growth over the past decade and is characterized by significant racial, ethnic, linguistic, and cultural diversity. These demographic characteristics influence healthcare access, HIV prevention needs, and the delivery of culturally responsive public health services.

Table 10. Selected Demographic Characteristics of Gwinnett County, Georgia

Indicator	Value
Population (2025)	1,018,099
Female Population (%)	50.9%
Black/African American Population (%)	31.2%
Hispanic/Latino Population (%)	24.5%
Asian Population (%)	14.8%
Foreign-Born Population (%)	28.4%
Language Other Than English Spoken at Home (%)	38.1%
Persons Without Health Insurance (%)	16.8%
Persons Living in Poverty (%)	10.4%
Median Household Income	\$87,890

Source: U.S. Census Bureau QuickFacts, Gwinnett County, Georgia (2025).

Key Findings

Gwinnett County is a rapidly growing and highly diverse community. Approximately one-third of residents identify as Black/African American, nearly one-quarter identify as Hispanic/Latino, and more than one-quarter of residents are foreign-born. Additionally, over one-third of residents speak a language other than English at home.

Although the county has a relatively high median household income, approximately 10.4% of residents live below the federal poverty level and 16.8% of individuals under age 65 lack health insurance coverage. These factors may influence access to healthcare, HIV prevention services, and continuity of care.

Implications

The demographic characteristics of Gwinnett County support the need for culturally responsive HIV prevention strategies that address linguistic diversity, healthcare access barriers, and health equity. HIV prevention programs should prioritize outreach, education, testing, and navigation services that are accessible to diverse populations, including Black/African American communities, Hispanic/Latino populations, immigrants, and uninsured individuals.

F. HIV-Related Mortality

Table 11. HIV/AIDS Mortality in Gwinnett County (2017–2024)

Year	HIV/AIDS Deaths	Death Rate per 100,000
2017	9	1.0
2018	9	1.0
2019	11	1.2
2020	14	1.5
2021	7	0.7
2022	20	2.1
2023	12	1.2
2024	9	0.9
Total / Average	91	1.2

Source: Georgia Department of Public Health OASIS, HIV/AIDS Mortality Data.

Key Findings

Between 2017 and 2024, Gwinnett County reported 91 HIV/AIDS-related deaths, corresponding to an average death rate of 1.2 deaths per 100,000 population. Annual HIV/AIDS deaths fluctuated during the study period, reaching a high of 20 deaths in 2022 before declining to 9 deaths in 2024.

Implications

Although HIV-related mortality has declined substantially since the introduction of effective antiretroviral therapy, HIV/AIDS-related deaths continue to occur within Gwinnett County. These findings highlight the importance of early HIV diagnosis, timely linkage to care, retention in treatment, and viral suppression efforts to improve long-term outcomes for people living with HIV.

G. Community Assets

Despite ongoing HIV-related challenges, Gwinnett County possesses substantial community resources that support HIV prevention, testing, treatment, and care.

Table 12. Community Assets Identified

Asset Category	Examples
Community-Based Organizations	H.Y.P.E. to Empower
Public Health Agencies	Georgia Department of Public Health
Healthcare Providers	Positive Impact Health Centers
Regional Networks	Georgia North Rural Ending the HIV Epidemic Network
Community Outreach Programs	Local HIV education and prevention initiatives

Key Findings

The presence of established HIV prevention programs, healthcare providers, public health agencies, and community-based organizations creates a strong foundation for addressing HIV-related disparities and improving access to prevention and care services.

Implications

Building upon existing community assets and strengthening partnerships may improve service coordination, increase community engagement, expand prevention outreach, and support long-term sustainability of HIV prevention efforts throughout the region.

1.5 Identified Service Gaps and Priority Needs

The Community Needs Assessment identified several HIV prevention and care gaps within Gwinnett County. These gaps were derived from analysis of HIV prevalence, new HIV diagnoses, linkage-to-care outcomes, PrEP coverage trends, and demographic disparities. Addressing these needs is essential to reducing HIV-related disparities and advancing health equity among populations disproportionately affected by HIV.

Table 13. Service Gaps and Programming Implications

Gap	Description	Implication for Programming
Persistent HIV disparities among Black/African American and Hispanic/Latino populations	Black/African American residents experienced an HIV prevalence rate of 928.6 per 100,000 population, while Hispanic/Latino residents experienced a prevalence rate of 559.7 per 100,000 population.	Prioritize culturally responsive HIV prevention, education, and outreach efforts targeting disproportionately affected populations.
Linkage-to-care outcomes remain below national targets	Linkage to HIV medical care improved to 82.8% in 2022 but remains below the Ending the HIV Epidemic target of 95%.	Strengthen referral coordination, patient navigation, and follow-up support to improve timely engagement in HIV care.
PrEP utilization remains insufficient relative to HIV burden	PrEP coverage increased over time but remained at 29.8% in 2023 despite continued HIV transmission and elevated HIV burden.	Expand PrEP education, awareness campaigns, navigation services, and provider referral partnerships.
Continued HIV transmission within Gwinnett County	Gwinnett County reported 235 new HIV diagnoses in 2023, indicating ongoing transmission.	Increase access to community-based HIV testing, prevention education, and risk-reduction services.
Social and structural barriers influence HIV prevention and care access	HIV disparities continue despite the availability of prevention and treatment services, suggesting barriers related to healthcare access, stigma, transportation, and social determinants of health.	Develop partnerships and supportive services that address barriers to prevention, testing, and healthcare engagement.

Priority Needs

Based on the findings of this assessment, the following priority needs were identified:

- Increase access to culturally responsive HIV prevention and testing services among populations disproportionately affected by HIV.
- Expand PrEP awareness, education, and navigation services to improve utilization among individuals at elevated risk for HIV.
- Improve linkage-to-care processes and patient navigation support to increase timely engagement in HIV medical care following diagnosis.
- Strengthen community-based outreach and education efforts to reduce HIV-related stigma and increase awareness of prevention resources.
- Enhance collaboration among public health agencies, healthcare providers, and community-based organizations to improve service coordination and continuity of care.

Implications

The identified service gaps demonstrate that, despite improvements in HIV prevention and care indicators, substantial disparities and unmet needs remain within Gwinnett County. Persistent HIV burden among Black/African American and Hispanic/Latino communities, linkage-to-care outcomes below national targets, and insufficient PrEP coverage indicate continued need for targeted prevention and support services.

Addressing these gaps will require coordinated efforts among public health agencies, healthcare providers, community organizations, and community members. The findings of this assessment support the continued implementation and expansion of community-based HIV prevention programs designed to improve access, reduce disparities, and advance health equity.

Limitations

This assessment relied primarily on publicly available surveillance, demographic, and mortality data. Secondary data sources may be subject to reporting delays, incomplete case ascertainment, and variations in data collection methods. Additionally, county-level indicators may not fully capture neighborhood-level disparities or emerging HIV prevention needs within specific communities.

1.6 Summary

The findings of this Community Needs Assessment demonstrate that HIV remains a significant public health issue within Gwinnett County. Analysis of surveillance data revealed a growing number of individuals living with HIV, continued new HIV diagnoses, and persistent disparities among Black/African American and Hispanic/Latino populations. Although progress has been made in improving linkage-to-care outcomes and expanding PrEP coverage, current indicators remain below national Ending the HIV Epidemic targets and suggest ongoing unmet prevention and care needs within the community.

The assessment also identified several priority service gaps, including persistent racial and ethnic disparities in HIV burden, insufficient PrEP utilization, continued HIV transmission, and barriers to timely engagement in care. These findings highlight the importance of community-based HIV prevention strategies that improve access to testing, prevention education, PrEP navigation, and linkage-to-care services. Addressing these gaps will require culturally responsive approaches that recognize the unique needs of populations disproportionately affected by HIV while also addressing broader social and structural determinants that influence health outcomes.

At the same time, the assessment identified substantial community assets that can support HIV prevention efforts, including public health agencies, healthcare providers, community-based organizations, regional partnerships, and existing HIV service networks. Leveraging these resources provides an opportunity to strengthen coordination of care, expand outreach efforts, and improve access to prevention and support services.

The Health & Linkage Services Program operated by H.Y.P.E. to Empower is well positioned to address several of the priority needs identified through this assessment. Through HIV testing, prevention education, PrEP navigation, community outreach, referral services, and partnership development, the program seeks to reduce HIV-related disparities and improve access to prevention resources among populations disproportionately affected by HIV. The following Program Evaluation Plan was developed to assess the implementation, reach, and outcomes of the Health & Linkage Services Program and to identify opportunities for continuous quality improvement and future program growth.

1.7 References

AIDSVu. (2025). *HIV prevalence and new diagnoses by county*. Rollins School of Public Health, Emory University. <https://aidsvu.org>

Arrington-Sanders, R., Hailey-Fair, K., Wirtz, A. L., Morgan, A., Brooks, D., Castillo, M., Trexler, C., Kwait, J., Dowshen, N., Galai, N., Beyrer, C., & Celentano, D. (2020). Role of Structural Marginalization, HIV Stigma, and Mistrust on HIV Prevention and Treatment Among Young Black Latinx Men Who Have Sex with Men and Transgender Women: Perspectives from Youth Service Providers. *AIDS Patient Care and STDs*, 34(1), 7–15. <https://doi.org/10.1089/apc.2019.0165>

Furukawa NW, Maksut JL, Zlotorzynska M, Sanchez TH, Smith DK, Baral SD. Sexuality Disclosure in U.S. Gay, Bisexual, and Other Men Who Have Sex With Men: Impact on Healthcare-Related Stigmas and HIV Pre-Exposure Prophylaxis Denial. *Am J Prev Med*. 2020 Aug;59(2):e79-e87. doi: 10.1016/j.amepre.2020.02.010. Epub 2020 May 4. PMID: 32376144; PMCID: PMC7375915.

Georgia Department of Public Health. (2025). *HIV surveillance summary, Georgia*. HIV Epidemiology Section, Georgia Department of Public Health. <https://dph.georgia.gov>

Hussen SA, Wade RM, Newman A Jr, Alohan DI, Harper GW. Conceptualizations of Mental Health Among Young Black Gay, Bisexual, and Other Men (YB-GBMSM) Who Have Sex With Men Who Are Living With HIV. *AIDS Educ Prev*. 2024 Dec;36(6):387-402. doi: 10.1521/aeap.2024.36.6.387. PMID: 39705174; PMCID: PMC12902678.

Mann LM, Kelley CF, Siegler AJ, Stephenson R, Sullivan PS. Seroadaptive Strategy Patterns of Young Black Gay, Bisexual, and Other Men Who Have Sex With Men in Atlanta, Georgia. *J Acquir Immune Defic Syndr*. 2022 Jan 1;89(1):40-48. doi: 10.1097/QAI.0000000000002816. PMID: 34878433; PMCID: PMC8711639.

Piske M, Nosyk B, Smith JC, Yeung B, Enns B, Zang X, Sullivan PS, Armstrong WS, Thompson MA, Daniel G, Del Rio C. Ending the HIV Epidemic in Metropolitan Atlanta: a mixed-methods study to support the local HIV/AIDS response. *J Int AIDS Soc*. 2024 Jul;27(7):e26322. doi: 10.1002/jia2.26322. PMID: 39039716; PMCID: PMC11263453.

Saldana, C., Philpott, D. C., Mauck, D. E., Hershow, R. B., Garlow, E., Gettings, J., Freeman, D., France, A. M., Johnson, E. N., Ajmal, A., Elimam, D., Reed, K., Sulka, A., Adame, J. F., Andía, J. F., Gutierrez, M., Padilla, M., Jimenez, N. G., Hayes, C., ... Wortley, P.. (2023). Public Health Response to Clusters of Rapid HIV Transmission Among Hispanic or Latino Gay, Bisexual, and Other Men Who Have Sex with Men — Metropolitan Atlanta, Georgia, 2021–2022. *MMWR. Morbidity and Mortality Weekly Report*, 72(10), 261–264. <https://doi.org/10.15585/mmwr.mm7210a3>

U.S. Department of Health and Human Services. (2022). *National HIV/AIDS strategy for the United States: 2022–2025*. Office of Infectious Disease and HIV/AIDS Policy.<https://files.hiv.gov/s3fs-public/NHAS-2022-2025.pdf>

U.S. Department of Health and Human Services, Centers for Disease Control and Prevention. (2023). *HIV surveillance report, 2022* (Vol. 34).
<https://www.cdc.gov/hiv/library/reports/hiv-surveillance.html>

Appendices

Appendix A

Full Literature Review

Until It Ends: Assessing HIV-Related Community Needs and Evaluating Community-Based HIV Prevention and Linkage Services

Link: <https://drive.google.com/file/d/1QCShH0yBL3A8-harfYKOA-jq-xLsZMXW/view?usp=sharing>